

Q&A LOG
Healthcare Delivery Solution (HCDS) Electronic Health Record (EHR) Follow On (MHS GENESIS)
Industry Day 31 March 2026

QUESTION #	PROSPECTIVE VENDOR'S QUESTION	GOVERNMENT'S RESPONSE
1	Will industry be able to adjust pricing based on this extension?	The Government will negotiate the price of services and solutions with the prime contractor. The Government will not be involved in negotiations between prime and sub-contractors.
2	Will the government allow reuse of existing ATO artifacts to accelerate onboarding for incumbent technologies like Dentrax Enterprise?	The ATOs will not be impacted by the contract changes. The current ATOs will remain in place, but if the way the solution is delivered to the Government changes, the ATO must be updated.
3	If the Government moves to a direct license/maintenance or SaaS purchasing model while also prohibiting direct subcontracts with product vendors, how will the direct-sale construct work in practice, and what contractual/vendor qualification criteria (including any defense-contractor seller requirements) will vendors be expected to meet to enable those purchases?	Please review the DOD Implementation Guide of Executive Order (EO) 14222 https://media.defense.gov/2025/May/28/2003725174/-1/-1/1/MEMORANDUM-DIRECTING-IMPLEMENTATION-OF-EXECUTIVE-ORDER-14222-DEPARTMENT-OF-GOVERNMENT-EFFICIENCY-COST-EFFICIENCY-INITIATIVE.PDF
4	Funding / scope: How much of the annual contract for the various efforts is expected to be funded when the new contract is established? How can subcontractors best project near term scope and resource requirements?	At this time, The Government cannot provide specific funding amounts.
5	Where are costs/efforts captured for supporting unique government upgrade protocols such as the block upgrade process?	Please review slides 27-32 for Governance. The Government anticipates each solution provider will coordinate with the Government prior to performing system upgrades or patches. There will also be support required from other contractors (e.g. technical integration, end user support, etc.) depending upon the upgrade or patch. Governance is going to be key.
6	Considering performance-based compensation, will they employ T&M/CPFF contracts for development of new capabilities?	This "follow-on" strategy is only applicable to current solutions.
7	Will the strategy have pathways for SBIR, direct to phase 3 technologies?	This would be incumbent on the company to demonstrate a SBIR phase 3 technology would apply to the requirement as part of the proposal back to the Government.
8	Do you plan to leverage approaches such as demos to ensure products/solutions are able to perform as proposed?	The Government is currently developing evaluation criteria to ensure we can effectively assess all proposed solutions.
9	How have you mitigate the knowledge transfer risk previously cited around the technical documentation of the system?	Currently additional technical documentation is in development. That documentation along with the updated contract strategy affords Government along with the incumbent contractor to have a more robust transition strategy to support knowledge transfer.
10	Will there be multiple award for each LOE	Each requirement discussed will have its own contracting approach/ strategy.
11	What additional "remaining solutions" are you considering for this fall?	Please refer to Slide 11

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12	Would you share more about the line between the training responsibilities of the solution providers and the end user service provider. Who is responsible for what?	The solution provider is responsible for providing to the commercial training material for the capabilities, to include updates to the commercial training materials. The End User Services is responsible for ensuring the end users are able to maximize the value of the system to its fullest extent. The solution providers may be incentivized to train Help Desk Tier 1/ Tier 2 contractors to ensure
13	Anything you can share regarding target contract vehicles or CSO or basic contracts sam.gov announcement) for these efforts?	Each requirement discussed will be posted on sam.gov.
14	How does the integration into dha change your peo enterprise services. IE wf 3.0?	The PEO DHMS integration with DHA should have no impact on the contract strategy presented.
15	Will there be an expectation to manage this work via Agile/SAFe Agile/etc? Or up to the contractor?	Each contractor must propose their approach to meeting the outcomes identified.
16	Are you using Agile Program management or SAFe (Scaled Agile Framework)?	Each contractor must propose their approach to meeting the outcomes identified.
17	In pursuit of competition and fresh ideas, Will the government waive the need for PPQs as commercial customers are hesitant to complete them	For each requirement, the Government will review the FAR/DFARS requirements and develop the appropriate strategy.
18	How does the CSO recently released and rereleased play into the construct that you displayed today? Seems to be overlap with integration.	The Government acknowledges your question concerning the potential overlap with the active CSO. As that is an open solicitation, we are unable to address any questions related.
19	Operational Medicine in austere environments — Will the contract require hands-free or voice-activated documentation; how will this be evaluated in HCDS selection?	Each contractor must propose an approach to meet the outcomes identified.
20	Do you anticipate your pending integration into DHA bringing any new risks to your schedule?	No
21	Are you able to identify/disclose the solutions you plan to sole source other than Oracle, Philips & Amwell?	Please refer to Slide 11
22	What's the new length of solution based contracts	The Government is evaluating a 1-year base with four (4) 1-year options for a total of five years.
23	When outcomes are impacted by cross-functional dependencies, how is that reflected in performance evaluation?	The QASP is where the Government will formally define how it measures performance. It will include metrics that assess the integrated team's ability to deliver on shared outcomes
24	How does DHA intend to measure value and outcomes for dental including Dentrix-class capabilities, separately from acute EHR metrics; ensure dental data is accurately reflected in enterprise and joint analytics; and decide whether dental will remain bundled in larger awards or may be modular / separately competed so dental vendors are judged on dental outcomes and lifecycle cost?	Outcomes will be defined and agreed upon with each solution provider.
25	Is the Deployment RFP different from PEO DHMS Deployment Solutions RFP that includes Fed EHR and OpMed?	No, these are the same requirement.

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26	Not seeing mention of DHIO. What is the plan to merge DHMS with DHA/DHIO to leverage current contracts providing similar services such as end user support. Things such as EHRU/Adoption Central/Adoption/Change mngt teams.	The Government plans to merge the end user support requirement under DHIO.
27	What might be the requirements related to COI and if an Offeror can bid and win on multiple Service contracts? Can a small sub be proposed on multiple teams?	In accordance with FAR 9.505, all offerors are responsible for identifying any potential or actual Organizational Conflicts of Interest (OCI). If an OCI is anticipated, the proposal must include a comprehensive mitigation plan.
28	Does the "shift left" strategy for help desk imply direct informatics steering committee support at the site level?	Direct informatics steering committee support is not a requirement as part of this contract strategy.
29	Are all of the service contracts going to be MACs?	See response to #10
30	For the OEMs with a Deployment Requirement, is it the expectation that there will be two different RFPs...a sole-source and a one under Deployment?	No, the Government intends to award a multiple award IDIQ for deployment services.
31	'Strategy and coordination above the line. Execution below the line.' I'm watching the disaster this is creating in JOMIS. Different APM and product managers (above the line) focused on making sure their products deliver on time with no cross talk on technical integration. Different standards are being used for messaging and communications, high level governance of terms and shared services are not in place because it is no one's job.	Noted. See slides 27-32 to review the Governance Model DHMSM is or has implemented to ensure communication for integration is occurring. This does not apply to JOMIS
32	Is there flexibility in consideration of interface engine options that work offer greater efficiency and savings to the government?	The offeror is responsible for providing a technical approach to the contractual requirement to meet the objectives, outcomes, SLAs, etc.
33	Will the Government provide a standardized data-sharing framework so sub-system partners can validate value metrics, particularly when outcomes depend on interoperability with the broader MHS GENESIS core?	Yes, the Government will provide data sharing and interoperability.
34	What expectations does DHA have for automated monitoring, patching, and lifecycle management?	See slides # 27 for Governance. These items will also be addressed as part of the contractual outcomes/objectives.
35	How do you balance technical integration with forcing a solution provider to change core parts of their solution - not because of a true security issue, but because the government architecture team starts to put their own preferences and vision into a commercial solution?	The Government does not intend to require a company to change its COTS solution due to a Government preference.
36	Will government staff visual designers with HCD to leverage prototypes and mockups?	The offeror is responsible for providing a technical approach to the requirement that meets the objectives, outcomes, SLAs, etc.
37	How are you reconciling the goal of continuous improvement and not having a vendor identify requirements and offer improvements (as mentioned in the OCI discussion)?	The Government defines the requirements, and vendors propose solutions to meet those needs.